

## ABOUT THIS PORTFOLIO

# Document Overview

This portfolio (Rev. 1.2, August 2026) contains the fifteen written plans, policies, and procedures referenced in §12.1–§12.19 of the **Well Spring Intervention LLC SOP Manual (Rev. 2.24)** required to demonstrate conformance to the 2026 CARF Child and Youth Services (CYS) Standards Manual at the time of an **Inaugural One-Year Accreditation survey**, per the **2026 CYS Inaugural Accreditation Guidelines** (effective July 1, 2026 – June 30, 2027). Each plan is a self-contained written policy with purpose, scope, policy statement, procedure, responsible parties, review schedule, and approval signature block. The QP maintains the signed originals in the facility compliance binder and presents this portfolio to CARF surveyors as the primary evidence of organizational readiness.

**Document ID.** Doc. WSI-CARF-PLANS-001, Rev. 1.2 (August 2026).

**Population Served.** Children & Adolescents — Mental Health / Behavioral Challenges.

**Service Type.** Staff Secure Group Home — 10A NCAC 27G.

**Service Intensity.** The Staff Secure Group Home provides 24-hour supervised residential care for children and adolescents with serious emotional disturbance who require structured clinical services and continuous staff supervision, but whose needs can be safely met without the hardware-secure physical plant and on-site intensive clinical staffing of a Level III Residential Treatment Facility. Features include staff-supervised egress (doors may be locked to manage elopement risk), continuous staff supervision, awake overnight staff, minimum 1:6 direct-care staff-to-resident ratio during waking hours and 1:8 overnight, Licensed Professional on-site during business hours and on-call 24/7, psychiatrist available per the Person-Centered Plan medication-management schedule, RN available per the health-services schedule and on-call for urgent medical needs, individual therapy minimum weekly, group therapy multiple times per week, family therapy as clinically indicated, and Person-Centered Plan reviews at minimum every 90 days or as clinically indicated.

**Document Owner.** Executive Director & Qualified Professional (QP).

**Companion Documents.** SOP Manual v2.24 (Doc. WSI-SOP-001, Rev. 2.24); Resident Handbook (Doc. WSI-RH-001); CARF CYS 2026 Compliance Audit Crosswalk (Doc. WSI-CARF-XW-001).

## Plan Portfolio Contents.

This portfolio contains fifteen plans, organized in the order referenced in SOP §12. Each plan is numbered consistently with the SOP §12 subsection that references it:

- **Plan 1 — Strategic Plan** (§12.1) — three-year strategic goals, mission/vision/values, environmental analysis, succession planning
- **Plan 2 — Stakeholder Input Plan** (§12.2) — quarterly surveys, advisory convening, "You Said / We Did" feedback loop, non-retaliation
- **Plan 3 — Legal Compliance Plan** (§12.3) — Compliance Officer, Legal Compliance Register, annual legal-compliance review
- **Plan 4 — Financial Plan** (§12.4) — annual budget, capital reserve, internal controls, audit, resident trust funds
- **Plan 5 — Enterprise Risk Management Plan** (§12.5) — Enterprise Risk Register, business-continuity plan, incident integration

- **Plan 6 — Health & Safety Committee Charter** (§12.6) — standing committee, monthly meetings, written health-and-safety policies
- **Plan 7 — Workforce Development Plan** (§12.7) — recruiting, orientation curriculum, annual training, position descriptions, performance evaluation
- **Plan 8 — Resident Rights Policy Compilation** (§12.8) — eleven rights, grievance procedure, audit and trend reporting
- **Plan 9 — Accessibility & Nondiscrimination Plan** (§12.9) — nondiscrimination, language access, reasonable accommodation, physical accessibility
- **Plan 10 — Performance Measurement Plan** (§12.10) — 25 KPIs across seven domains, data aggregation, quarterly Governing Body reporting
- **Plan 11 — Program Description** (§12.11) — populations served, service array, program philosophy, staffing, physical environment
- **Plan 12 — Screening and Access Policy** (§12.12) — referral intake, screening criteria, admission decision, waitlist, denial appeals
- **Plan 13 — Quality Records Review Procedure** (§12.17) — quarterly Form 8 audits, trend analysis, corrective action
- **Plan 14 — Telehealth & ICT Service Delivery Policy** (§12.18) — eligible services, technology platforms, informed consent, emergency procedures
- **Plan 15 — CARF CYS 2026 Sections 3–5 Standards Crosswalk** (§12.19) — 38-item crosswalk mapping standards to written policies

### **Approval & Implementation.**

Each plan includes an Approval signature block requiring signatures from the Executive Director, Clinical Director, Qualified Professional (QP), and Compliance Officer. Upon approval, each plan becomes a written policy of Well Spring Intervention LLC, effective upon the date of the last signature. Plans are reviewed at minimum annually and updated as needed to reflect changes in CARF standards, regulatory requirements, organizational structure, or program design.

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*This portfolio is organized into fifteen plans, each addressing a specific CARF CYS 2026 standard area referenced in SOP §12.1–§12.19. Each plan is self-contained and may be reviewed, approved, and implemented independently. The QP maintains the master copy of all fifteen plans in the compliance binder, with signed originals of each. A complete revision history appears at the end of this portfolio.*

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**CARF CYS 2026 CONFORMANCE PLANS**

# Written Policies, Plans & Procedures Portfolio

*This portfolio contains the fifteen written plans, policies, and procedures referenced in §12.1–§12.19 of the Well Spring Intervention LLC SOP Manual (Rev. 2.24) required to demonstrate conformance to the 2026 CARF Child and Youth Services (CYS) Standards Manual at the time of an Inaugural One-Year Accreditation survey. Each plan is a self-contained written policy with purpose, scope, policy statement, procedure, responsible parties, review schedule, and approval signature block. Plans are presented in the order they are referenced in §12. The QP maintains the signed originals in the facility compliance binder and presents this portfolio to CARF surveyors as the primary evidence of organizational readiness.*

## 1. Strategic Plan

*Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §12.1*

### 1.1 Purpose.

The Strategic Plan establishes the long-term direction of Well Spring Intervention LLC and demonstrates conformance to CARF CYS Section 1.A (Leadership) and Section 1.C (Strategic Planning). It documents the organization's mission, vision, and values; identifies the populations to be served and the services to be provided; analyzes the internal and external environment; sets measurable strategic goals over a three-year horizon; and assigns responsibility and resources for execution. The Strategic Plan is the primary evidence of leadership's intentional direction of the organization and is presented to the CARF survey team at the Inaugural Accreditation survey as the foundational governance document.

### 1.2 Scope.

This plan applies to the entire organization, including the Governing Body, Executive Director, Clinical Director, Qualified Professional (QP), all direct-care and administrative personnel, contractors, and volunteers. It governs strategic decisions related to program design, population served, service array, geographic service area, capital development, workforce composition, financial structure, and accreditation strategy. Tactical and operational decisions are governed by the SOP Manual and are not within the scope of this Strategic Plan.

### 1.3 Mission, Vision & Values.

**Mission.** Well Spring Intervention LLC provides trauma-informed, high-quality supervised living environments for youth with severe emotional disturbances (SED), ensuring their safety and clinical growth through individualized, evidence-based care.

**Vision.** Every child served by Well Spring Intervention LLC will experience a stable, predictable, and nurturing environment in which healing occurs, families are strengthened, and youth transition to less restrictive settings with the skills and supports needed to thrive.

**Values.** Safety · Dignity · Accountability · Growth · Empowerment · Freedom · Health · Wholeness · Healing. These values inform every personnel decision, clinical intervention, and operational practice.

### 1.4 Populations & Services.

The organization serves children and adolescents (target age range 6–17) with a primary diagnosis of mental illness, emotional disturbance, or substance-related disorder, who do not meet inpatient psychiatric criteria but require removal from the home and treatment in a staff-secure group home setting. The service array includes: 24-hour supervised residential care in a Staff Secure Group Home licensed under 10A NCAC 27G; clinical services including individual therapy (minimum weekly), group therapy (multiple times per week), family therapy (as clinically indicated, minimum biweekly), and clinical assessments; psychiatric medication management with psychiatric consultation per the Person-Centered Plan schedule and on-call psychiatric consultation for urgent clinical concerns; behavioral support and crisis intervention with continuous staff supervision; educational coordination through the local public school system or facility-based instructional programming; case management and care coordination with the LME/MCO and other system-of-care partners; and structured recreational, social, and life-skills programming integrated into the daily activity schedule.

### 1.5 Environmental Analysis.

The Strategic Plan incorporates an annual environmental analysis considering input from persons served (via the Stakeholder Input Plan, Plan 2), personnel (via the annual engagement survey), funders (via Alliance Health and NCDHHS communications), referral sources (via quarterly referral-pattern review), and the community (via the annual community advisory convening). The analysis identifies opportunities (e.g., emerging Medicaid benefit categories, community partnerships, workforce pipeline programs) and threats (e.g., workforce shortages, reimbursement rate changes, regulatory shifts, competitive entrants) over the three-year horizon.

### 1.6 Three-Year Strategic Goals (2026–2028).

**Goal 1 — Inaugural CARF Accreditation.** Achieve CARF Child & Youth Services Inaugural One-Year Accreditation by Q1 2027 and subsequent full Three-Year Accreditation by Q1 2028. Owner: Executive Director. Measure: Accreditation certificate on file; QIP submitted within 90 days of notification (Form 12).

**Goal 2 — Census Stabilization.** Achieve and sustain an average daily census of 90% of licensed capacity by Q4 2026, with no more than 10% of admissions resulting in unplanned discharge within 30 days. Owner: Executive Director & QP. Measure: Monthly census report to Governing Body; discharge-trend dashboard.

**Goal 3 — Workforce Stability.** Reduce personnel turnover to below 25% annually (industry benchmark) by Q4 2027, with 100% of direct-care personnel completing the orientation curriculum and annual refresher training per Plan 7. Owner: QP. Measure: Quarterly personnel engagement survey; monthly turnover report.

**Goal 4 — Clinical Outcomes Excellence.** Demonstrate measurable clinical improvement in 80% of youth served, as measured by validated symptom and functioning instruments (e.g., PHQ-A, GAD-7, CAFAS) at admission vs. discharge. Owner: Clinical Director. Measure: Quarterly outcomes report to Governing Body per Plan 10.

**Goal 5 — Financial Sustainability.** Maintain a minimum of 60 days cash-on-hand, achieve annual independent financial audit with no material findings, and sustain operating margins sufficient to fund the capital-replacement reserve per Plan 4. Owner: Executive Director. Measure: Quarterly financial report to Governing Body.

### 1.7 Action Plans & Evaluation.

Each strategic goal has a written action plan identifying the responsible party, required resources, timeline (quarterly milestones), and evaluation method. The QP tracks action-plan progress monthly and reports status to the Executive Director. The Governing Body reviews progress against the Strategic Plan at each quarterly meeting and authorizes revisions as needed. The Strategic Plan is reviewed and updated in full at least annually, with the annual revision incorporating the year's environmental analysis, performance data (Plan 10), stakeholder input (Plan 2), and any QIP actions (Plan 12 in SOP §12.20).

### 1.8 Succession Planning.

The Strategic Plan includes a written succession plan for the Executive Director, Clinical Director, and QP positions. For each role, the succession plan identifies: (a) the position's core competencies and credentialing requirements; (b) internal candidates and their development needs; (c) external recruitment strategy and pipeline; (d) interim coverage procedures in the event of sudden vacancy; and (e) knowledge-transfer and documentation requirements. The succession plan is reviewed annually and following any key leadership change.

### 1.9 Responsible Parties.

**Executive Director** — overall accountability for the Strategic Plan; chairs the strategic planning process; presents the plan to the Governing Body. **Governing Body** — reviews, approves, and monitors the Strategic Plan at each quarterly meeting. **QP** — coordinates data collection, environmental analysis, and progress tracking; maintains the Strategic Plan in the compliance binder. **Clinical Director** — provides clinical program input and outcome data. **Compliance Officer** — ensures alignment with CARF, regulatory, and accreditation requirements.

### 1.10 Review Schedule.

Annual full review and revision (due: July 1 of each year, synchronized with the CARF standards-manual effective date). Quarterly progress reviews at each Governing Body meeting. Ad-hoc revision triggered by: significant environmental change, serious incident, accreditation finding, regulatory change, or leadership transition.

### Plan Approval

The undersigned approve this **Strategic Plan** as a written policy of Well Spring Intervention LLC, effective upon the date of the last signature below. This plan shall be reviewed at minimum annually and updated as needed to reflect changes in CARF standards, regulatory requirements, organizational structure, or program design. The QP maintains the signed original in the facility compliance binder and provides a copy to all personnel with implementation responsibilities.

| Role                        | Name (Printed) | Signature | Date |
|-----------------------------|----------------|-----------|------|
| Executive Director          |                |           |      |
| Clinical Director           |                |           |      |
| Qualified Professional (QP) |                |           |      |
| Compliance Officer          |                |           |      |

## 2. Stakeholder Input Plan

*Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §12.2*

### 2.1 Purpose.

The Stakeholder Input Plan demonstrates conformance to CARF CY5 Section 1.D by establishing written methods for gathering, analyzing, and acting on input from persons served, their families/Legally Responsible Persons (LRPs), personnel, referral sources, payers, and the community. The plan ensures that the organization's strategic direction, program design, and service delivery are informed by the voices of those most affected by its work, and that the organization communicates back to stakeholders how their input was used.

### 2.2 Scope.

This plan covers all input-gathering activities conducted by the organization, including surveys, focus groups, advisory convenings, grievance data, complaint logs, and informal feedback channels. It applies to all personnel who interact with stakeholders, including direct-care staff, clinical staff, the QP, the Executive Director, and Governing Body members.

### 2.3 Input Methods.

The organization uses at least four structured methods for gathering stakeholder input:

**Quarterly Youth Satisfaction Survey.** Developmentally appropriate, anonymous, offered in English and the youth's preferred language; covers safety, respect, food, activities, school, clinical services, family contact, and overall satisfaction. Administered by the QP or designee; results aggregated by facility and trended quarterly.

**Semi-Annual Family/LRP Feedback Survey.** Distributed electronically and on paper; covers communication, visitation, treatment planning participation, satisfaction with clinical services, and recommendations. Results shared with the Clinical Director and the CFT.

**Annual Personnel Engagement Survey.** Anonymous; covers supervision, training, safety, workload, communication, leadership, and intent to remain. Results presented to the Governing Body and inform the Workforce Development Plan (Plan 7).

**Annual Community/Stakeholder Advisory Convening.** Open invitation to LME/MCO representatives, school personnel, DSS, juvenile justice, former residents and families (with consent), faith community, and neighborhood residents. Facilitated discussion of program strengths, gaps, and opportunities; minutes documented and posted.

### 2.4 Input Aggregation & Analysis.

The QP aggregates all input data at least quarterly, analyzes for themes and trends, and prepares a written summary for the Governing Body. The summary identifies: (a) the input methods used; (b) the number of respondents per method; (c) the key themes; (d) the actions taken or planned in response; and (e) any unresolved issues requiring Governing Body direction. Quantitative survey results are presented with comparison to prior periods to identify trends.

### 2.5 "You Said / We Did" Feedback Loop.

The organization communicates back to stakeholders how their input was used through visible "You Said / We Did" postings in the facility common area (updated monthly), in resident community meetings (weekly), in family newsletters (quarterly), and in personnel meetings (monthly). This closed-loop feedback is essential to maintaining stakeholder trust and demonstrating that input is valued and acted upon.

### 2.6 Use of Input.

Stakeholder input directly informs: (a) the annual revision of the Strategic Plan (Plan 1); (b) the Person-Centered Plan for each youth (SOP §4.1); (c) program structure and service array decisions (Plan 11); (d) the Workforce Development Plan (Plan 7); (e) the Performance Measurement Plan (Plan 10); and (f) the Quality Improvement Plan submitted to CARF (SOP §12.20, Form 12).

## 2.7 Non-Retaliation.

The organization prohibits retaliation against any person served, family member, LRP, personnel member, or other stakeholder for providing input, filing a grievance, or reporting a concern. The non-retaliation policy is published in the Resident Handbook, the personnel handbook, and posted in the facility. Personnel who engage in retaliation are subject to disciplinary action up to and including termination. The QP investigates any allegation of retaliation within 5 business days.

## 2.8 Responsible Parties.

**QP** — overall coordination of input-gathering activities, data aggregation, analysis, and reporting. **Executive Director** — convenes the annual community advisory convening; presents input summary to Governing Body. **Governing Body** — reviews input summary at each quarterly meeting; directs action. **All Personnel** — encourage stakeholder input, do not retaliate, communicate the "You Said / We Did" feedback.

## 2.9 Review Schedule.

Quarterly review of input data and aggregation. Annual review of the Stakeholder Input Plan itself (method effectiveness, response rates, representation). Ad-hoc review following any grievance trend, serious incident, or significant stakeholder concern.

## Plan Approval

The undersigned approve this **Stakeholder Input Plan** as a written policy of Well Spring Intervention LLC, effective upon the date of the last signature below. This plan shall be reviewed at minimum annually and updated as needed to reflect changes in CARF standards, regulatory requirements, organizational structure, or program design. The QP maintains the signed original in the facility compliance binder and provides a copy to all personnel with implementation responsibilities.

| Role                        | Name (Printed) | Signature | Date |
|-----------------------------|----------------|-----------|------|
| Executive Director          |                |           |      |
| Clinical Director           |                |           |      |
| Qualified Professional (QP) |                |           |      |
| Compliance Officer          |                |           |      |

### 3. Legal Compliance Plan

*Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §12.3*

#### 3.1 Purpose.

The Legal Compliance Plan demonstrates conformance to CARF CYS Section 1.E by establishing a written framework for identifying, monitoring, and complying with all applicable federal, state, and local legal requirements governing the organization and the program. The plan designates a Compliance Officer responsible for monitoring legal requirements and coordinating the annual legal-compliance review.

#### 3.2 Scope.

This plan applies to all legal and regulatory requirements applicable to the organization's operations, including licensure, Medicaid provider enrollment, health-care fraud and abuse laws, health-privacy law, child-mandated-reporter law, disability-rights law, civil-rights law, employment law, and zoning/fire/building/life-safety codes. It covers written policies, monitoring activities, response to legal inquiries, and reporting of potential violations.

#### 3.3 Compliance Officer Designation.

The Executive Director designates a Compliance Officer (may be the QP) responsible for: (a) monitoring changes in legal requirements; (b) maintaining the Legal Compliance Register (Section 3.5); (c) coordinating the annual legal-compliance review; (d) investigating reports of potential violations; (e) reporting identified violations to the Governing Body within 5 business days of discovery; and (f) coordinating with legal counsel. The Compliance Officer reports directly to the Executive Director and has access to the Governing Body as needed.

#### 3.4 Applicable Legal Requirements.

The organization maintains written policies addressing compliance with the following categories of legal requirements:

**Licensure.** Staff Secure Group Home license issued by the NC Department of Health and Human Services under 10A NCAC 27G (SOP §1.2); renewed prior to expiration; changes in ownership, capacity, population, or location require prior written approval. The Staff Secure Group Home level of care serves children and adolescents with serious emotional disturbance who require 24-hour supervised residential care with structured clinical services and continuous staff supervision, but who do not require the intensive, hardware-secure level of care provided in a Level III Residential Treatment Facility.

**Medicaid Provider Enrollment.** Enrollment with NCTracks and Alliance Health; revalidation per CMS schedule; compliance with Medicaid fraud and abuse laws (False Claims Act, Anti-Kickback Statute, Physician Self-Referral Law) (SOP §10).

**Health Privacy.** HIPAA Privacy, Security, and Breach Notification Rules; HITECH Act; 42 CFR Part 2 (SUD records confidentiality); state law on AIDS/confidential conditions (SOP §11).

**Child Mandated Reporting.** State law requiring all personnel to report suspected child abuse/neglect to DSS; written reporting procedure (SOP §8).

**Disability Rights.** Americans with Disabilities Act (ADA) Title III; Section 504 of the Rehabilitation Act; state disability-rights law (Plan 9).

**Civil Rights.** Title VI of the Civil Rights Act; Age Discrimination Act; state civil-rights law (Plan 9).

**Employment Law.** Fair Labor Standards Act; Family and Medical Leave Act; Equal Employment Opportunity laws; OSHA; state employment law; workers' compensation.

**Life-Safety Codes.** NFPA 101 Life Safety Code; state fire code; building code; zoning ordinance (SOP §9).

**Accreditation.** CARF CYS 2026 Standards Manual; 2026 CYS Inaugural Accreditation Guidelines (SOP §12).

### 3.5 Legal Compliance Register.

The Compliance Officer maintains a written Legal Compliance Register listing each applicable legal requirement, the controlling authority, the responsible position, the most recent review date, the next review due date, and the status. The Register is reviewed monthly by the Compliance Officer and presented to the Governing Body at each quarterly meeting. The Register is the source-of-truth document for all legal compliance activities.

### 3.6 Annual Legal-Compliance Review.

The Compliance Officer coordinates an annual legal-compliance review, engaging legal counsel as needed. The review covers each item in the Legal Compliance Register, verifies current compliance, identifies any gaps, and documents corrective actions. The written annual review report is presented to the Executive Director and the Governing Body, with an attestation of compliance signed by the Executive Director and Clinical Director.

### 3.7 Response to Legal Inquiries.

The organization maintains a written procedure for responding to subpoenas, court orders, government investigations, and media inquiries. The procedure requires: (a) immediate notification of the Executive Director and Compliance Officer; (b) notification of legal counsel within 24 hours; (c) preservation of all relevant records; (d) coordination of any response through legal counsel; (e) documentation of the inquiry and response in the compliance binder; and (f) reporting to the Governing Body at the next regular meeting (or sooner if material).

### 3.8 Reporting Potential Violations.

Any personnel member who becomes aware of a potential legal violation must report it to the Compliance Officer within 24 hours. The Compliance Officer investigates each report within 5 business days, documents findings, and reports confirmed violations to the Executive Director and Governing Body within 5 business days of confirmation. The organization does not retaliate against any personnel member for good-faith reporting of a potential violation. Personnel may also report concerns anonymously through the compliance hotline.

### 3.9 Responsible Parties.

**Executive Director** — designates the Compliance Officer; signs the annual compliance attestation. **Compliance Officer** — maintains the Legal Compliance Register; coordinates the annual review; investigates reports. **QP** — operational compliance for clinical and program requirements. **Governing Body** — reviews compliance reports; ensures corrective action. **Legal Counsel** — provides legal advice; reviews legal inquiries.

### 3.10 Review Schedule.

Monthly review of the Legal Compliance Register. Annual full legal-compliance review with counsel. Ad-hoc review triggered by regulatory change, serious incident, or legal inquiry.

## Plan Approval

The undersigned approve this **Legal Compliance Plan** as a written policy of Well Spring Intervention LLC, effective upon the date of the last signature below. This plan shall be reviewed at minimum annually and updated as needed to reflect changes in CARF standards, regulatory requirements, organizational structure, or program design. The QP maintains the signed original in the facility compliance binder and provides a copy to all personnel with implementation responsibilities.

| Role                        | Name (Printed) | Signature | Date |
|-----------------------------|----------------|-----------|------|
| Executive Director          |                |           |      |
| Clinical Director           |                |           |      |
| Qualified Professional (QP) |                |           |      |
| Compliance Officer          |                |           |      |

## 4. Financial Plan

*Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §12.4*

### 4.1 Purpose.

The Financial Plan demonstrates conformance to CARF CY5 Section 1.F by establishing written policies and procedures for financial planning, budgeting, internal controls, audit, billing, and records retention. The plan ensures the organization maintains financial sustainability sufficient to deliver quality services and meet its obligations to persons served, personnel, funders, and the community.

### 4.2 Scope.

This plan covers all financial activities of the organization, including operating budget, capital budget, cash management, resident trust funds, billing and collections, payroll, accounts payable, procurement, financial reporting, audit, and records retention. It applies to the Executive Director, QP, Billing Coordinator, and any personnel with financial responsibilities.

### 4.3 Annual Operating Budget.

The Executive Director prepares an annual operating budget covering all revenue sources (Medicaid, room-and-board, grants, charitable contributions) and all expense categories (personnel, facility, clinical, administrative). The budget is reviewed and approved by the Governing Body prior to the start of the fiscal year. Material variances (greater than 10% line-item variance) are documented with explanation and corrective action. The budget is reviewed at each quarterly Governing Body meeting.

### 4.4 Capital-Replacement Reserve.

The organization maintains a capital-replacement reserve sufficient to address expected major repairs and replacements over a rolling five-year horizon. The reserve is funded through an annual allocation in the operating budget (minimum 3% of operating revenue). The QP maintains a written five-year capital-replacement schedule listing each major asset, expected useful life, replacement cost, and target replacement year. The reserve is held in a separate bank account and is used only for capital purposes.

### 4.5 Internal Controls.

Written internal controls address: (a) segregation of duties between the person authorizing payments, the person processing payments, and the person reconciling bank statements; (b) authorization thresholds for expenditures (e.g., under \$500 — QP; \$500–\$5,000 — Executive Director; over \$5,000 — Governing Body); (c) monthly bank reconciliation by an individual not authorized to sign checks; (d) segregation of resident trust funds from operating funds in a separate bank account with monthly reconciliation and quarterly reporting to each resident and LRP; (e) dual signatures on all checks over \$1,000; (f) monthly review of expense reports by the Executive Director; and (g) annual review of internal controls by the auditor.

### 4.6 Resident Trust Funds.

The organization maintains a written resident trust-fund policy addressing: (a) deposits (intake of personal funds at admission, receipt of SSI/SSDI benefits, gifts); (b) withdrawals (consistent with the youth's Person-Centered Plan, with LRP authorization for amounts over \$50); (c) interest (any interest earned is credited to the youth's account); (d) quarterly accounting to the youth and LRP; (e) reconciliation at discharge with full disbursement of remaining funds; and (f) protection from unauthorized access. The QP conducts monthly audit of resident trust-fund transactions.

### 4.7 Independent Financial Audit.

The organization engages an independent certified public accountant (CPA) to conduct an annual financial audit (or reviewed financial statements for organizations below the audit threshold). The audit engagement letter is approved by the Governing Body. The auditor issues a written report including the opinion, financial statements, and any management-letter findings. The Executive Director prepares a written response to any findings, presented to the Governing Body within 60 days of receipt. The audit report is retained permanently.

#### **4.8 Billing & Collections.**

Written billing and collections policies are maintained consistent with SOP §10.9 and Medicaid requirements. The Billing Coordinator submits claims within 5 business days of service delivery, monitors claim status weekly, follows up on denials within 10 business days, and reports monthly to the QP on claim volume, denial rate, and aging. The QP coordinates with clinical staff to ensure service documentation supports billed claims (SOP §10). Write-offs over \$500 require Executive Director approval; write-offs over \$5,000 require Governing Body approval.

#### **4.9 Financial Records Retention.**

Financial records are retained per the written records-retention schedule, which is consistent with state law, Medicaid requirements, and IRS requirements. At minimum: tax returns — permanent; audit reports — permanent; bank statements — 7 years; accounts payable and accounts receivable — 7 years; payroll records — 7 years; resident trust-fund records — 7 years after discharge. Records are stored securely with access limited to authorized personnel.

#### **4.10 Financial Reporting.**

The Executive Director prepares a quarterly financial report for the Governing Body including: (a) balance sheet; (b) income statement (actual vs. budget); (c) cash-flow statement; (d) days-cash-on-hand calculation; (e) accounts-receivable aging; (f) resident trust-fund balance; and (g) capital-replacement reserve balance. The report is presented at each quarterly Governing Body meeting and retained in the compliance binder.

#### **4.11 Responsible Parties.**

**Executive Director** — overall financial accountability; prepares budget; presents to Governing Body. **Governing Body** — approves budget; reviews quarterly financial reports; engages auditor. **QP** — coordinates billing oversight and resident trust-fund audit. **Billing Coordinator** — daily billing operations; weekly claim-status monitoring. **Compliance Officer** — ensures internal controls are followed.

#### **4.12 Review Schedule.**

Monthly internal financial review (Executive Director + QP). Quarterly Governing Body financial review. Annual independent audit. Annual full review of the Financial Plan itself.

### **Plan Approval**

The undersigned approve this **Financial Plan** as a written policy of Well Spring Intervention LLC, effective upon the date of the last signature below. This plan shall be reviewed at minimum annually and updated as needed to reflect changes in CARF standards, regulatory requirements, organizational structure, or program design. The QP maintains the signed original in the facility compliance binder and provides a copy to all personnel with implementation responsibilities.

| Role                        | Name (Printed) | Signature | Date |
|-----------------------------|----------------|-----------|------|
| Executive Director          |                |           |      |
| Clinical Director           |                |           |      |
| Qualified Professional (QP) |                |           |      |
| Compliance Officer          |                |           |      |

## 5. Enterprise Risk Management Plan

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §12.5

### 5.1 Purpose.

The Enterprise Risk Management (ERM) Plan demonstrates conformance to CARF CY5 Section 1.G by establishing a written framework for identifying, assessing, mitigating, and monitoring risks across the organization. The plan includes the Enterprise Risk Register, business-continuity plan, and the procedures for integrating risk management into governance, operations, and incident response.

### 5.2 Scope.

This plan covers all categories of risk facing the organization, including clinical, operational, financial, legal/compliance, reputational, and external risks. It applies to the Governing Body, Executive Director, Compliance Officer, QP, and all personnel with risk-management responsibilities.

### 5.3 Risk-Management Governance.

The Compliance Officer serves as the risk-management lead, reporting directly to the Executive Director with access to the Governing Body as needed. The Compliance Officer: (a) maintains the Enterprise Risk Register; (b) coordinates the annual risk-management review; (c) reviews serious incidents within one business day; (d) trends incidents monthly; (e) presents risk-management reports to the Governing Body at each quarterly meeting; and (f) coordinates the business-continuity plan and annual drill.

### 5.4 Enterprise Risk Register.

The Enterprise Risk Register identifies at minimum the following risk categories, with each risk scored on likelihood (1–5) and impact (1–5), and assigned a mitigation action, responsible position, and review date:

| Risk Category | Specific Risk               | Likelihood (1-5) | Impact (1-5) | Mitigation Action                                                                                                                                                                                                                                             | Owner              | Review Date |
|---------------|-----------------------------|------------------|--------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------|-------------|
| Clinical      | Suicide attempt / self-harm | 3                | 5            | Columbia suicide-risk screening at admission & weekly; safety plan; line-of-sight supervision as needed; staff training                                                                                                                                       | Clinical Director  | Quarterly   |
| Clinical      | Elopement from facility     | 3                | 4            | Staff-secure group home with continuous staff supervision; awake overnight staff; elopement risk assessment at admission & weekly; staff protocol for missing-resident response; community-search procedure; police notification within 30 min if not located | QP                 | Quarterly   |
| Clinical      | Restraint-related injury    | 2                | 5            | De-escalation training; restraint-avoidance policy; post-incident debriefing (Form 3); quarterly restraint review                                                                                                                                             | Clinical Director  | Quarterly   |
| Clinical      | Medication error            | 3                | 4            | Med-pass training; 6-month regimen review (Form 11); med-error reporting; double-check for high-alert meds                                                                                                                                                    | RN / QP            | Monthly     |
| Operational   | Staffing shortage           | 4                | 4            | Recruitment plan; PRN pool; agency contract backup; sign-on incentive; retention bonuses                                                                                                                                                                      | Executive Director | Monthly     |

| Risk Category    | Specific Risk                        | Likelihood (1-5) | Impact (1-5) | Mitigation Action                                                                                                | Owner              | Review Date |
|------------------|--------------------------------------|------------------|--------------|------------------------------------------------------------------------------------------------------------------|--------------------|-------------|
| Operational      | Capacity under-utilization           | 3                | 3            | Referral-source relationships; LME/MCO engagement; marketing; admission-decision responsiveness                  | Executive Director | Monthly     |
| Operational      | IT system failure (EHR)              | 2                | 4            | Daily off-site backup; paper-fallback procedures (SOP §10.7); 72-hour transcription SLA; vendor support contract | QP                 | Quarterly   |
| Financial        | Medicaid audit finding               | 2                | 5            | Monthly service-note audit (Form 8); quarterly QP review; annual external documentation audit; staff training    | QP                 | Quarterly   |
| Financial        | Payer-mix concentration              | 3                | 3            | Diversification strategy; grant-seeking; charitable fundraising; cost-rate analysis                              | Executive Director | Annual      |
| Financial        | Cash-flow shortfall                  | 2                | 5            | 60-day cash reserve; line of credit; monthly cash-flow projection; accelerated billing                           | Executive Director | Monthly     |
| Legal/Compliance | Privacy breach (PHI)                 | 2                | 5            | HIPAA training; access controls; encryption; breach-response plan; annual risk assessment                        | Compliance Officer | Quarterly   |
| Legal/Compliance | Licensing deficiency                 | 2                | 5            | Mock survey annually; continuous compliance monitoring; corrective-action tracking                               | Compliance Officer | Annual      |
| Legal/Compliance | Employment claim                     | 2                | 4            | Personnel policies; EPLI insurance; documentation; supervisory training                                          | Executive Director | Annual      |
| Reputational     | Adverse media / community concern    | 2                | 4            | Crisis-communication plan; community engagement; transparent incident reporting                                  | Executive Director | Annual      |
| External         | Natural disaster (tornado/hurricane) | 2                | 5            | Emergency Operations Plan (Protocol 19); drill per shift quarterly; supply cache; evacuation plan                | QP                 | Quarterly   |
| External         | Public-health emergency (pandemic)   | 2                | 5            | Infection-control plan (Protocol 18); PPE supply; telehealth capability; visitor screening                       | RN / QP            | Quarterly   |

### 5.5 Business-Continuity Plan.

The organization maintains a written business-continuity plan coordinated with SOP §9 Emergency Operations Plan and Protocol 19. The plan addresses: (a) critical functions to be maintained during a disruption (resident care, medication administration, clinical services, billing); (b) alternate facility arrangements in the event the primary facility is uninhabitable; (c) communication procedures with personnel, families/LRPs, LME/MCO, and licensing authority; (d) IT system recovery procedures (daily off-site backup, vendor recovery SLA, paper-fallback); (e) essential-vendor list (pharmacy, medical, food, fuel); and (f) annual continuity-plan drill with after-action review.

### 5.6 Incident-Review Integration.

The Compliance Officer reviews every serious incident report (per SOP §8 IRIS) within one business day of submission. The review identifies any new risks not yet on the Enterprise Risk Register, updates likelihood/impact scores for existing risks as warranted, and ensures corrective actions are tracked to completion. Serious incidents are trended monthly by type, location, shift, and personnel. Trends are presented to the Health & Safety Committee (Plan 6) and to the Governing Body quarterly.

## 5.7 Annual Risk-Management Review.

The Compliance Officer coordinates an annual risk-management review presented to the Governing Body. The review includes: (a) summary of all risks on the Enterprise Risk Register with current scoring and mitigation status; (b) analysis of incident trends; (c) review of any near-misses; (d) evaluation of mitigation-action effectiveness; (e) identification of emerging risks; (f) revision of the Register as needed; and (g) recommendations for the coming year. The annual review is documented in writing and retained in the compliance binder.

## 5.8 Responsible Parties.

**Compliance Officer** — risk-management lead; maintains Register; coordinates annual review. **Executive Director** — overall accountability; allocates resources to mitigation. **Governing Body** — reviews Register at each quarterly meeting; directs major mitigation actions. **QP** — operational risk management for clinical/program risks. **RN** — clinical/medication risks.

## 5.9 Review Schedule.

Quarterly review of the Enterprise Risk Register by the Governing Body. Annual full risk-management review. Ad-hoc review following any serious incident, near-miss, or significant organizational change.

## Plan Approval

The undersigned approve this **Enterprise Risk Management Plan** as a written policy of Well Spring Intervention LLC, effective upon the date of the last signature below. This plan shall be reviewed at minimum annually and updated as needed to reflect changes in CARF standards, regulatory requirements, organizational structure, or program design. The QP maintains the signed original in the facility compliance binder and provides a copy to all personnel with implementation responsibilities.

| Role                        | Name (Printed) | Signature | Date |
|-----------------------------|----------------|-----------|------|
| Executive Director          |                |           |      |
| Clinical Director           |                |           |      |
| Qualified Professional (QP) |                |           |      |
| Compliance Officer          |                |           |      |

## 6. Health & Safety Committee Charter

*Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §12.6*

### 6.1 Purpose.

The Health & Safety Committee Charter demonstrates conformance to CARF CYS Section 1.H by establishing a standing committee responsible for ongoing oversight of health, safety, and environmental conditions in the facility. The committee reviews inspections, drills, incidents, infection-control surveillance, medication errors, and work-related injuries, and recommends corrective action to the Executive Director and Governing Body.

### 6.2 Scope.

The committee's scope covers all health and safety matters within the facility and during organization-sponsored activities off-site. This includes physical-plant safety, environmental health, infection control, emergency preparedness, drill scheduling and evaluation, incident review, medication-error review, workplace-violence prevention, and personnel safety training.

### 6.3 Committee Composition.

The standing members of the Health & Safety Committee are: (a) **QP** (chair, voting); (b) **House Manager** (voting); (c) **Direct Care Professional representative** (rotating among day/evening/overnight shifts, voting); (d) **RN or designee** (voting); (e) **Compliance Officer** (non-voting, advisory); and (f) **Executive Director** (ex officio, non-voting). The committee may invite additional subject-matter experts (facilities manager, IT coordinator, etc.) as needed. Membership is reviewed annually.

### 6.4 Meeting Frequency & Quorum.

The committee meets at least monthly, on the second Tuesday of each month at 1:00 PM. Special meetings may be called by the chair or any two voting members with at least 48 hours' notice. Quorum is a majority of voting members (3 of 4). Decisions are made by consensus where possible; otherwise by majority vote of voting members present. The chair does not vote except to break a tie.

### 6.5 Standing Agenda.

Each monthly meeting follows a standing agenda:

- Review and approval of prior meeting minutes
- Monthly environmental safety inspection results (Form 5)
- Fire, tornado, and lockdown drills per shift (§9.2)
- Incident reports trended by type, location, shift, and personnel (§8)
- Infection-control surveillance (§6.1, Protocol 18)
- Medication-error trends (§6.3)
- Work-related injury trends
- Equipment and physical-plant work orders
- Safety-plan revisions
- Emergency-preparedness plan revisions
- Old business / action-item follow-up
- New business

### 6.6 Minutes & Documentation.

The committee maintains written minutes documenting attendance, topics reviewed, decisions made, and action items with responsible parties and due dates. Minutes are distributed to all committee members within 5 business

days of the meeting, retained permanently in the compliance binder, and made available to CARF surveyors on request. Action items are tracked to completion at each subsequent meeting.

## 6.7 Reporting.

The QP (as chair) reports committee activities and trends to the Clinical Director at each quarterly compliance report (per SOP §1.4(a)) and to the Governing Body at each quarterly meeting. The report includes: (a) summary of inspections and drills; (b) incident trends; (c) infection-control surveillance; (d) medication-error trends; (e) work-related injury trends; (f) action-item status; and (g) recommendations for resource allocation or policy revision.

## 6.8 Written Health & Safety Policies.

Written health-and-safety policies are maintained in the facility policy library, addressing at minimum:

- Emergency response (fire, tornado, lockdown, evacuation, medical emergency)
- Infection control (hand hygiene, PPE, isolation, cleaning/disinfection, surveillance)
- Hazard communication (chemical inventory, SDS, labeling, training)
- Bloodborne pathogens (exposure control plan, Hepatitis B vaccination, post-exposure)
- Workplace violence prevention (warning signs, reporting, response)
- Hot work / environmental safety (facility-specific hazards)
- Food safety (kitchen sanitation, temperature monitoring, allergen control)
- Pool/water safety (if applicable)
- Vehicle and transport safety (if applicable)

Each policy is reviewed at least annually by the committee, with revision as needed to reflect changes in regulation, facility, or practice. Personnel are trained on each applicable policy at orientation and annually thereafter.

## 6.9 Responsible Parties.

**QP (chair)** — convenes meetings; sets agenda; tracks action items; reports to Governing Body. **House Manager** — physical-plant inspections; work-order coordination. **DCP representative** — frontline perspective; identifies operational safety concerns. **RN** — infection control; medication safety. **Compliance Officer** — regulatory alignment; risk integration. **Executive Director** — resource allocation; policy approval.

## 6.10 Review Schedule.

Monthly committee meetings. Annual review of the Charter itself, including membership, scope, and effectiveness. Ad-hoc meetings following any serious incident, environmental emergency, or regulatory inspection.

## Plan Approval

The undersigned approve this **Health & Safety Committee Charter** as a written policy of Well Spring Intervention LLC, effective upon the date of the last signature below. This plan shall be reviewed at minimum annually and updated as needed to reflect changes in CARF standards, regulatory requirements, organizational structure, or program design. The QP maintains the signed original in the facility compliance binder and provides a copy to all personnel with implementation responsibilities.

| Role               | Name (Printed) | Signature | Date |
|--------------------|----------------|-----------|------|
| Executive Director |                |           |      |

| Role                        | Name (Printed) | Signature | Date |
|-----------------------------|----------------|-----------|------|
| Clinical Director           |                |           |      |
| Qualified Professional (QP) |                |           |      |
| Compliance Officer          |                |           |      |

## 7. Workforce Development Plan

*Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §12.7*

### 7.1 Purpose.

The Workforce Development Plan demonstrates conformance to CARF CY5 Section 1.I by establishing written policies for recruiting, hiring, orienting, training, supervising, evaluating, and retaining personnel. The plan ensures the organization has a qualified, competent, and engaged workforce sufficient to deliver quality services to the persons served.

### 7.2 Scope.

This plan covers all personnel of the organization, including full-time, part-time, PRN, and temporary personnel; contractors with direct contact with persons served; and volunteers and interns. It addresses recruiting, hiring, orientation, annual training, position descriptions, performance evaluation, supervision, personnel records, succession, and engagement.

### 7.3 Recruiting & Hiring.

The organization maintains a written recruiting and hiring policy (SOP §2.2) addressing: (a) position descriptions for every role, consistent with SOP §2; (b) posting and recruitment strategy (online job boards, professional associations, referral incentives, partnerships with local colleges); (c) application review and interview procedures, including structured interview questions aligned with position competencies; (d) background-check requirements per SOP §2.3 (criminal, abuse/neglect registry, sex-offender registry, education verification, license verification, reference checks); (e) credential verification per SOP §1.4(b) and §2.2; (f) offer of employment contingent on satisfactory background and credential verification; and (g) written acknowledgment of personnel policies (Form 6 — Employee SOP Acknowledgment).

### 7.4 Orientation Curriculum.

Every new personnel member completes a written orientation curriculum prior to working independently with persons served. The orientation curriculum covers at minimum:

- Mission, vision, and values of the organization
- Person-centered and trauma-informed care principles
- Person-Centered Plan process (SOP §4.1)
- Behavioral support and restraint policies (SOP §5)
- Medication administration policies (SOP §6.3) — for personnel who administer medications
- Infection control (SOP §6.1, Protocol 18)
- Incident reporting (SOP §8 — IRIS)
- Privacy and confidentiality (SOP §11 — HIPAA, 42 CFR Part 2)
- Cultural and linguistic competency
- Mandated reporting of suspected child abuse/neglect
- Emergency procedures (fire, tornado, lockdown, evacuation)
- Documentation requirements (SOP §10)
- Resident rights (Plan 8)
- Personnel code of conduct
- Workplace violence prevention
- Bloodborne pathogens / exposure control
- Vehicle and transport safety (if applicable)

Orientation is conducted by the QP or designee and is documented in the personnel file. A new-hire may not work independently with persons served until orientation is complete and the QP has verified competency.

### **7.5 Annual Training Plan.**

The QP maintains a written annual training plan covering refresher training and competency verification for all personnel. The plan identifies required training topics, frequency (annual, biennial, or as needed), target completion dates, trainers, and documentation method. Required annual training includes at minimum: trauma-informed care refresher; CPR/First Aid (biennial); bloodborne pathogens; HIPAA refresher; mandated reporter refresher; crisis prevention/intervention (CPI or equivalent) refresher; medication administration refresher (for med-pass personnel); and any topic identified through incident trends, performance evaluation, or regulatory change. The QP tracks training completion in the master training calendar and individual personnel files.

### **7.6 Position Descriptions.**

Written position descriptions are maintained for every role in the organization, including: Executive Director, Clinical Director, QP, Associated Professional (AP), Direct Care Professional (DCP), House Manager, Registered Nurse (RN), Billing Coordinator, Cook/Household Manager, and any other role. Each position description includes: (a) position title; (b) reports-to; (c) summary of responsibilities; (d) essential duties; (e) minimum qualifications (education, experience, credentials); (f) physical requirements; (g) working conditions; and (h) acknowledgment signature. Position descriptions are reviewed annually and updated to reflect changes in scope.

### **7.7 Performance Evaluation.**

Written performance-evaluation policy requires annual performance evaluation for every personnel member, conducted by the supervisor. The evaluation includes: (a) review of position description and essential duties; (b) competency assessment against position-specific criteria; (c) review of training completion; (d) review of any incidents or complaints; (e) self-assessment by the personnel member; (f) supervisor feedback; (g) goals for the coming year; and (h) signatures. New personnel receive a 90-day and 180-day evaluation in addition to the annual cycle. Evaluations are documented in the personnel file.

### **7.8 Supervision.**

Written supervision policy consistent with SOP §2.2(a) Individualized Supervision Plans. Each AP and DCP has a written individualized supervision plan specifying the frequency and format of supervision (individual, group, on-the-job observation), the supervisor, and the documentation method. The QP supervises clinical staff per the Clinical Director's direction. The House Manager supervises DCPs on shift. Supervision is documented in the personnel file with date, topics discussed, and any action items.

### **7.9 Personnel Records.**

Written personnel-records policy consistent with SOP §2.4. The QP maintains personnel files in a locked cabinet (or secured EHR) with access limited to the personnel member, the supervisor, the QP, the Executive Director, and authorized auditors. Files include at minimum: application, resume, position description acknowledgment, background-check results, credential verification, orientation documentation, training records, performance evaluations, supervision notes, and any disciplinary actions. Personnel files are retained for 7 years after separation.

### **7.10 Volunteer & Intern Management.**

Written volunteer and intern management policy per SOP §1.8. Volunteers and interns with direct contact with persons served complete a modified orientation, background check, and supervision plan. Volunteers and interns do not administer medications, conduct clinical interventions, or work unsupervised with persons served. A volunteer/intern coordinator (may be the QP) maintains records and schedules.

### 7.11 Continuing Education Support.

The organization supports continuing education for personnel through: (a) paid time off for required licensure continuing education; (b) tuition reimbursement for job-related coursework (up to \$2,000 per personnel member per year, subject to budget approval); (c) in-service training sessions; (d) access to online training platforms; and (e) conference attendance (with Executive Director approval). Continuing-education completion is documented in the personnel file.

### 7.12 Succession Planning.

Written succession plan for the Executive Director, Clinical Director, and QP positions is maintained as part of the Strategic Plan (Plan 1, Section 1.8). For each role, the plan identifies core competencies, internal candidates, external recruitment strategy, interim coverage procedures, and knowledge-transfer requirements. The succession plan is reviewed annually and following any key leadership change.

### 7.13 Workforce Engagement & Recognition.

The organization maintains a written workforce engagement and recognition plan including: (a) annual personnel engagement survey (per Plan 2); (b) monthly personnel meetings with structured agenda; (c) peer-recognition program; (d) employee-of-the-quarter award; (e) annual personnel appreciation event; (f) wellness initiatives; and (g) exit interviews to identify retention opportunities. Engagement data is incorporated into the Workforce Development Plan annual revision.

### 7.14 Responsible Parties.

**QP** — overall coordination of workforce development; orientation curriculum; annual training plan; personnel records; supervision of clinical staff. **Executive Director** — recruiting strategy; succession planning; resource allocation. **House Manager** — DCP supervision; scheduling. **Clinical Director** — clinical supervision direction; clinical competency verification. **Governing Body** — reviews workforce data; approves compensation structure.

### 7.15 Review Schedule.

Monthly review of training completion and turnover data. Quarterly review by the Governing Body. Annual full review of the Workforce Development Plan, incorporating engagement survey results and turnover analysis.

### Plan Approval

The undersigned approve this **Workforce Development Plan** as a written policy of Well Spring Intervention LLC, effective upon the date of the last signature below. This plan shall be reviewed at minimum annually and updated as needed to reflect changes in CARF standards, regulatory requirements, organizational structure, or program design. The QP maintains the signed original in the facility compliance binder and provides a copy to all personnel with implementation responsibilities.

| Role                        | Name (Printed) | Signature | Date |
|-----------------------------|----------------|-----------|------|
| Executive Director          |                |           |      |
| Clinical Director           |                |           |      |
| Qualified Professional (QP) |                |           |      |

| Role               | Name (Printed) | Signature | Date |
|--------------------|----------------|-----------|------|
| Compliance Officer |                |           |      |

## 8. Resident Rights Policy Compilation

*Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §12.8*

### 8.1 Purpose.

The Resident Rights Policy Compilation demonstrates conformance to CARF CY5 Section 1.J by consolidating the organization's written policies on the rights of persons served into a single reference document. The policies are also published in the Resident Handbook (companion document) and in SOP §§3, 5, 6, 8, 9, and 11; this compilation serves as the comprehensive reference for personnel, persons served, families/LRPs, and CARF surveyors.

### 8.2 Scope.

This compilation covers all rights of persons served, including dignity, participation in services, confidentiality, access to records, grievance procedures, freedom from unnecessary restraint and seclusion, informed consent, religious freedom, personal-funds management, and advance notification of discharge. It applies to all personnel and to all youth served by the organization.

### 8.3 Statement of Rights.

Every youth served by Well Spring Intervention LLC has the right to:

**Dignity and respect.** Be treated with dignity, respect, and humane care at all times, free from discrimination on the basis of race, color, national origin, religion, sex, gender identity or expression, sexual orientation, age, disability, or any other protected classification.

**Privacy.** Reasonable privacy in personal care, communications, visitation, and personal space; privacy of personal mail and telephone communications (consistent with safety and the Person-Centered Plan).

**Participation in service planning.** Participate in the development and review of the Person-Centered Plan (SOP §4.1); receive a copy of the plan; and refuse services (with documented consequences of refusal).

**Confidential communication and visitation.** Receive and send mail; receive visitors (subject to safety and the Person-Centered Plan); and communicate with family, attorneys, advocates, and clergy.

**Access to records.** Access one's own service record per SOP §11.5; receive a copy upon request; and appeal any restriction of access.

**File a grievance.** File a grievance without retaliation; receive a written acknowledgment within 2 business days; receive a written response within 30 calendar days; and appeal to the Executive Director and external advocacy resources.

**Freedom from unnecessary restraint and seclusion.** Be free from restraint and seclusion except as a last-resort safety intervention per SOP §5; receive post-incident debriefing (Form 3); and have restraint use reviewed by the QP and Clinical Director.

**Informed consent.** Give informed consent (or have the LRP give consent) for treatment and medication, with explanation of benefits, risks, alternatives, and the right to refuse.

**Religious freedom.** Exercise religious freedom; receive reasonable accommodation for religious practices; and not have any religious belief imposed.

**Personal funds management.** Have personal funds managed per SOP §1.8 and Plan 4; receive quarterly accounting; and receive full disbursement at discharge.

**Advance notification of discharge.** Receive advance written notification of any discharge or transfer per SOP §3.4(a); participate in transition planning per SOP §3.4 and §3.6; and appeal an involuntary discharge.

### 8.4 Grievance Procedure.

The organization maintains a written grievance procedure. Any youth, family member, LRP, or other stakeholder may file a grievance orally or in writing with any personnel member. The personnel member receiving the grievance documents it on the grievance log and forwards it to the QP within 24 hours. The QP (or designee) acknowledges

the grievance in writing within 2 business days, investigates, and provides a written response within 30 calendar days. If the grievant is not satisfied with the response, they may appeal to the Executive Director within 10 business days; the Executive Director issues a written decision within 15 business days. The grievant is also informed of external advocacy resources, including the LME/MCO, the state licensing authority, Disability Rights NC, and the CARF complaint process. The QP maintains the grievance log, trends grievances quarterly, and reports to the Governing Body.

### 8.5 Personnel Training.

All personnel are trained on resident rights at orientation and annually thereafter. Training covers: (a) the statement of rights; (b) the grievance procedure; (c) the non-retaliation policy; (d) informed consent; (e) freedom from restraint and seclusion; (f) confidentiality; and (g) reporting rights violations. Training completion is documented in the personnel file.

### 8.6 Audit & Trend Reporting.

The QP audits the grievance log, restraint log, and rights-related incident reports quarterly. Findings are trended and reported to the Clinical Director, Executive Director, and Governing Body. Trends inform personnel training, supervision, and policy revision. Serious or repeated rights violations are reported to the state licensing authority per regulatory requirements.

### 8.7 Responsible Parties.

**QP** — overall management of resident rights; grievance investigation; quarterly audit. **Clinical Director** — clinical-care rights; restraint review. **All Personnel** — respect resident rights; report violations; do not retaliate. **Governing Body** — reviews trends; ensures corrective action.

### 8.8 Review Schedule.

Quarterly audit of grievance log, restraint log, and rights-related incidents. Annual review of the Resident Rights Policy Compilation itself. Ad-hoc review following any serious rights violation or regulatory change.

### Plan Approval

The undersigned approve this **Resident Rights Policy Compilation** as a written policy of Well Spring Intervention LLC, effective upon the date of the last signature below. This plan shall be reviewed at minimum annually and updated as needed to reflect changes in CARF standards, regulatory requirements, organizational structure, or program design. The QP maintains the signed original in the facility compliance binder and provides a copy to all personnel with implementation responsibilities.

| Role                        | Name (Printed) | Signature | Date |
|-----------------------------|----------------|-----------|------|
| Executive Director          |                |           |      |
| Clinical Director           |                |           |      |
| Qualified Professional (QP) |                |           |      |
| Compliance Officer          |                |           |      |



## 9. Accessibility & Nondiscrimination Plan

*Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §12.9*

### 9.1 Purpose.

The Accessibility & Nondiscrimination Plan demonstrates conformance to CARF CYS Section 1.K by establishing written policies ensuring that persons served, family members, personnel, and visitors are not subjected to discrimination and that the program is accessible to persons with disabilities and persons with limited English proficiency.

### 9.2 Scope.

This plan covers all aspects of the organization's operations, including admission, service delivery, employment, facility accessibility, communication, and grievance procedures. It applies to all personnel and to all persons seeking or receiving services.

### 9.3 Nondiscrimination Policy.

The organization does not discriminate on the basis of race, color, national origin, religion, sex, gender identity or expression, sexual orientation, age, disability, veteran status, or any other classification protected by applicable federal, state, or local law. This policy applies to admission, treatment, employment, and all other organizational activities. The policy is published in the Resident Handbook, the personnel handbook, on the organization's website, and posted in the facility. The Compliance Officer is designated as the Section 504 Coordinator and the Civil Rights Coordinator.

### 9.4 Language Access Plan.

The organization maintains a written language access plan providing for: (a) qualified interpreter services at no cost to the youth and family, available within 60 minutes for spoken languages and within 4 hours for sign language; (b) translation of vital documents (intake forms, consent forms, Resident Handbook, discharge summary, grievance procedure) into the languages commonly encountered in the service area (at minimum Spanish); (c) primary-language preference documented in the clinical record; (d) language-assistance need identified at intake; (e) use of qualified interpreters rather than family members or untrained personnel, except in emergencies; (f) tracking of language-assistance utilization for resource planning; and (g) annual review of language-access needs based on census data.

### 9.5 Reasonable Accommodation Policy.

The organization provides reasonable accommodation to youth, family members, personnel, and visitors with disabilities, consistent with the ADA and Section 504. Accommodations may include: (a) physical accessibility modifications (ramps, grab bars, accessible bathrooms); (b) communication accommodations (large-print materials, screen readers, captioning, auxiliary aids); (c) programmatic accommodations (modified schedules, modified activities, assistive technology); and (d) dietary accommodations (medical-diagnosis-driven). Requests for accommodation are made to the QP or Compliance Officer, documented in writing, evaluated within 5 business days, and implemented within 15 business days (or sooner if urgently needed). Denials are documented with rationale and appeal rights.

### 9.6 Physical Accessibility.

The organization conducts a written accessibility review of the physical facility at least annually, using the ADA Checklist for Existing Facilities. The review identifies barriers and corrective actions, with priority given to barriers affecting program access. The review is documented and maintained in the compliance binder. Any new

construction or alteration complies with the ADA Standards for Accessible Design. The QP maintains an accessibility-barrier removal plan with target dates and budget.

### 9.7 Nondiscrimination Grievance Procedure.

The organization maintains a written grievance procedure for nondiscrimination complaints, distinct from but coordinated with the resident grievance procedure in Plan 8. The procedure includes: (a) submission of the complaint to the Compliance Officer (Section 504 Coordinator); (b) written acknowledgment within 5 business days; (c) investigation within 30 calendar days; (d) written decision with rationale; (e) appeal to the Executive Director within 10 business days; (f) Executive Director decision within 15 business days; and (g) notice of external appeal rights (OCR, EEOC, state human relations commission). The procedure is published in the Resident Handbook and the personnel handbook.

### 9.8 Personnel Training.

All personnel are trained on nondiscrimination, language access, and reasonable accommodation at orientation and annually thereafter. Training covers: (a) the nondiscrimination policy; (b) language-access procedures; (c) reasonable-accommodation procedures; (d) cultural and linguistic competency; (e) the grievance procedure; and (f) the role of the Section 504 Coordinator. Training completion is documented in the personnel file.

### 9.9 Responsible Parties.

**Compliance Officer** — Section 504 Coordinator; Civil Rights Coordinator; investigates grievances. **QP** — operational implementation; intake screening; accommodation evaluation. **Executive Director** — resource allocation; appeals. **Governing Body** — reviews accessibility data; ensures barrier-removal funding.

### 9.10 Review Schedule.

Annual review of the Accessibility & Nondiscrimination Plan, including the physical accessibility review, language-access utilization, and grievance trends. Ad-hoc review following any regulatory change, demographic shift, or grievance.

### Plan Approval

The undersigned approve this **Accessibility & Nondiscrimination Plan** as a written policy of Well Spring Intervention LLC, effective upon the date of the last signature below. This plan shall be reviewed at minimum annually and updated as needed to reflect changes in CARF standards, regulatory requirements, organizational structure, or program design. The QP maintains the signed original in the facility compliance binder and provides a copy to all personnel with implementation responsibilities.

| Role                        | Name (Printed) | Signature | Date |
|-----------------------------|----------------|-----------|------|
| Executive Director          |                |           |      |
| Clinical Director           |                |           |      |
| Qualified Professional (QP) |                |           |      |
| Compliance Officer          |                |           |      |



## 10. Performance Measurement Plan

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §12.10

### 10.1 Purpose.

The Performance Measurement Plan demonstrates conformance to CARF CY5 Section 1.L by establishing the organization's performance indicators, data-collection methods, analysis cadence, and reporting structure. The plan ensures the organization collects, analyzes, and acts on data to improve services and outcomes for persons served.

### 10.2 Scope.

This plan covers all performance indicators tracked by the organization, organized into seven domains: access; person-centered planning; clinical outcomes; safety; personnel; financial; and person-served experience. It applies to the QP (data aggregation), all personnel (data collection), the Executive Director (resource allocation), and the Governing Body (review and oversight).

### 10.3 Performance Indicators.

The organization tracks the following performance indicators, with target values and reporting frequency:

| Domain   | Indicator                                     | Target                     | Frequency | Data Source           | Owner              |
|----------|-----------------------------------------------|----------------------------|-----------|-----------------------|--------------------|
| Access   | Time from referral to admission decision      | ≤ 5 business days          | Monthly   | Referral log          | QP                 |
| Access   | Time from admission to first clinical contact | ≤ 72 hours                 | Monthly   | Clinical record       | Clinical Director  |
| Access   | No-show rate for scheduled appointments       | ≤ 10%                      | Monthly   | Scheduling system     | QP                 |
| PCP      | PCP completed within required timeframe       | 100%                       | Monthly   | Clinical record audit | QP                 |
| PCP      | PCP reviewed at required intervals            | 100%                       | Monthly   | Clinical record audit | QP                 |
| PCP      | Youth/family participation in PCP meetings    | ≥ 90%                      | Quarterly | PCP signature page    | Clinical Director  |
| Outcomes | Symptom improvement (PHQ-A / GAD-7)           | ≥ 80% show improvement     | Quarterly | Validated instruments | Clinical Director  |
| Outcomes | Functional improvement (CAFAS)                | ≥ 70% show improvement     | Quarterly | CAFAS scoring         | Clinical Director  |
| Outcomes | Restraint frequency                           | ≤ 0.5 per 1,000 youth-days | Monthly   | Restraint log         | QP                 |
| Outcomes | Elopement frequency                           | ≤ 0.3 per 1,000 youth-days | Monthly   | Incident reports      | QP                 |
| Outcomes | Incident frequency and severity               | Trend analysis             | Monthly   | IRIS reports          | Compliance Officer |
| Safety   | Incident-free days                            | Trending upward            | Monthly   | IRIS reports          | Compliance Officer |
| Safety   | Time-to-investigation                         | ≤ 1 business day           | Monthly   | IRIS reports          | Compliance Officer |
| Safety   | Repeat-incident rate                          | ≤ 10%                      | Quarterly | IRIS reports          | Compliance Officer |

| Domain     | Indicator                            | Target              | Frequency   | Data Source          | Owner               |
|------------|--------------------------------------|---------------------|-------------|----------------------|---------------------|
| Safety     | Medication-error rate                | ≤ 1 per 1,000 doses | Monthly     | MAR / med-error log  | RN                  |
| Personnel  | Turnover rate (annual)               | ≤ 25%               | Quarterly   | Personnel records    | QP                  |
| Personnel  | Time-to-fill open positions          | ≤ 45 days           | Monthly     | Recruiting log       | Executive Director  |
| Personnel  | Training completion rate             | 100%                | Monthly     | Training calendar    | QP                  |
| Personnel  | Personnel engagement score           | ≥ 4.0 / 5.0         | Annual      | Engagement survey    | Executive Director  |
| Financial  | Days cash-on-hand                    | ≥ 60 days           | Monthly     | Financial statements | Executive Director  |
| Financial  | Medicaid denial rate                 | ≤ 5%                | Monthly     | Billing system       | Billing Coordinator |
| Financial  | Cost per youth per day               | Trend analysis      | Quarterly   | Financial statements | Executive Director  |
| Experience | Youth satisfaction score             | ≥ 4.0 / 5.0         | Quarterly   | Satisfaction survey  | QP                  |
| Experience | Family/LRP satisfaction score        | ≥ 4.0 / 5.0         | Semi-annual | Satisfaction survey  | QP                  |
| Experience | Complaint volume and resolution time | Trend analysis      | Quarterly   | Grievance log        | QP                  |

#### 10.4 Data Collection & Aggregation.

Each indicator has a designated data source and owner responsible for data collection. The QP aggregates data monthly, calculates the indicator value, and compares to the target. Data is stored in a master performance-measurement spreadsheet (or EHR analytics module) with monthly time-stamped entries. Data quality is verified quarterly through spot-checks against source records.

#### 10.5 Analysis & Reporting.

The QP analyzes performance data monthly for trends, identifies indicators that are off-target, and prepares a written quarterly report for the Governing Body. The report includes: (a) current value for each indicator; (b) trend over time (with comparison to prior quarters); (c) indicators that are off-target; (d) analysis of root causes; (e) recommended corrective actions; and (f) status of prior corrective actions. The report is presented at each quarterly Governing Body meeting.

#### 10.6 Use of Performance Data.

Performance data is used to: (a) inform the annual revision of the Strategic Plan (Plan 1); (b) identify areas for quality improvement (incorporated into the QIP per SOP §12.20); (c) allocate resources (personnel, training, capital); (d) recognize and reinforce positive performance; and (e) demonstrate accountability to stakeholders. Performance data is shared with personnel and with persons served (in developmentally appropriate, aggregated form) at least quarterly through the "You Said / We Did" feedback loop (Plan 2).

#### 10.7 Responsible Parties.

**QP** — overall coordination; data aggregation; quarterly report. **Clinical Director** — clinical outcomes data.

**Compliance Officer** — safety data. **RN** — medication data. **Billing Coordinator** — financial data. **Executive Director** — personnel and overall financial data; resource allocation. **Governing Body** — reviews quarterly report; directs corrective action.

### 10.8 Review Schedule.

Monthly data aggregation. Quarterly analysis and Governing Body report. Annual full review of the Performance Measurement Plan, including indicator selection, target values, and data-collection methods.

### Plan Approval

The undersigned approve this **Performance Measurement Plan** as a written policy of Well Spring Intervention LLC, effective upon the date of the last signature below. This plan shall be reviewed at minimum annually and updated as needed to reflect changes in CARF standards, regulatory requirements, organizational structure, or program design. The QP maintains the signed original in the facility compliance binder and provides a copy to all personnel with implementation responsibilities.

| Role                        | Name (Printed) | Signature | Date |
|-----------------------------|----------------|-----------|------|
| Executive Director          |                |           |      |
| Clinical Director           |                |           |      |
| Qualified Professional (QP) |                |           |      |
| Compliance Officer          |                |           |      |

## 11. Program Description

*Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §12.11*

### 11.1 Purpose.

The Program Description demonstrates conformance to CARF CY5 Section 2.A by documenting the populations served, the service array, the program philosophy, staffing patterns, physical environment, hours of operation, referral and intake process, cultural and linguistic competency plan, coordination with external systems of care, and the Person-Centered Plan process. The Program Description is approved by the Governing Body and serves as the primary evidence of the organization's program design for CARF surveyors.

### 11.2 Populations Served.

The program serves children and adolescents (target age range 6–17) with a primary diagnosis of mental illness, emotional disturbance, or substance-related disorder, who do not meet inpatient psychiatric criteria but require removal from the home and treatment in a staff-secure group home setting. The program is designed to serve youth with serious emotional disturbance who require 24-hour supervised residential care with structured clinical services and continuous staff supervision, but who do not require the intensive, hardware-secure level of care provided in a Level III Residential Treatment Facility. Admission criteria are documented in SOP §3.1 and the Screening and Access Policy (Plan 12).

### 11.3 Service Array.

The program provides the following services:

**Residential care** — 24-hour supervised living in a Staff Secure Group Home licensed under 10A NCAC 27G (SOP §9);

**Clinical services** — individual therapy, group therapy, family therapy, and clinical assessments (SOP §4);

**Psychiatric medication management** — psychiatric evaluation, medication management, 6-month drug-regimen review (SOP §6.3);

**Behavioral support** — behavioral assessment, behavioral support plan, crisis intervention, de-escalation, and (as last-resort) restraint (SOP §5);

**Educational coordination** — coordination with the local public school system, facility-based school programming when needed, IEP coordination (SOP §7);

**Case management** — care coordination with LME/MCO, DSS, juvenile justice, medical home, and other system-of-care partners;

**Recreational, social, and life-skills programming** — structured daily activities, community integration, life-skills curriculum (SOP §5.4);

**Family engagement** — family therapy, family visitation, family education, and transition support.

### 11.4 Program Philosophy.

The program is grounded in trauma-informed care, which recognizes that the vast majority of youth served have experienced significant trauma and that all interactions must be conducted in a manner that promotes safety, trustworthiness, choice, collaboration, and empowerment. The program is also person-centered, meaning that each youth's treatment is individualized based on their strengths, needs, goals, and preferences, as documented in the Person-Centered Plan (SOP §4.1). The program is committed to the use of evidence-based and evidence-informed practices, including cognitive-behavioral therapy, trauma-focused CBT, motivational interviewing, and positive behavior support.

### 11.5 Staffing Patterns.

The program is staffed per SOP §2, including: Executive Director, Clinical Director, Qualified Professional (QP), Associated Professionals (APs), Direct Care Professionals (DCPs) on day, evening, and awake-overnight shifts, House Manager, Registered Nurse (RN), and Billing Coordinator. Staffing ratios comply with the Staff Secure Group Home operating standards under 10A NCAC 27G, including a minimum 1:6 direct-care staff-to-resident ratio during waking hours, and a minimum 1:8 ratio overnight with at least one awake DCP at all times. A Licensed Professional (LP) is on-site during business hours and on call 24/7 for clinical emergencies. A psychiatrist is available for medication-management appointments per the Person-Centered Plan schedule and is on call for urgent psychiatric consultation. A Registered Nurse (RN) is available on-site per the medication-management and health-services schedule and on call for urgent medical needs. The QP provides clinical supervision per §1.4(a) and §2.2(a). A Licensed Professional provides minimum 4 hours per week face-to-face clinical consultation per §4.6 and Form 10.

### **11.6 Physical Environment.**

The facility is a free-standing residential facility designed and licensed as a Staff Secure Group Home under 10A NCAC 27G for children and adolescents. Physical-plant requirements are documented in SOP §9. The facility includes: resident bedrooms (single or double occupancy per licensing standards); communal dining and living areas; kitchen and food-storage areas; clinical offices and therapy rooms; a quiet room / de-escalation room (not used for seclusion — seclusion is prohibited under the organization's restraint-and-seclusion-minimization policy per SOP §5); medication storage area (double-locked per SOP §6.3(c)); laundry facilities; outdoor recreation area; and administrative offices. Staff-secure features include continuous staff supervision of residents, staff-monitored entry and exit (doors may be locked to elopement risk with staff-controlled release), door and window alarms as appropriate to the resident population, and a controlled visitor-entry process. The facility complies with NFPA 101 Life Safety Code, ADA accessibility standards, and state fire/building codes applicable to staff-secure group homes.

### **11.7 Hours of Operation.**

The program operates 24 hours per day, 7 days per week, 365 days per year. Clinical services are provided Monday–Friday 8:00 AM to 6:00 PM, with on-call clinical coverage outside those hours. Educational services are provided Monday–Friday during the school year per the local public school calendar. Recreational and life-skills programming is provided daily, including weekends.

### **11.8 Referral & Intake Process.**

Referrals are accepted from LME/MCOs, DSS, juvenile justice, inpatient psychiatric facilities, families/LRPs, and other system-of-care partners. The referral and intake process is documented in SOP §3 and the Screening and Access Policy (Plan 12). The QP screens each referral against admission criteria, communicates the admission decision within 5 business days, and coordinates intake logistics with the referring party.

### **11.9 Cultural & Linguistic Competency.**

The program is committed to cultural and linguistic competency, as documented in the Accessibility & Nondiscrimination Plan (Plan 9). Personnel receive annual cultural-competency training. The program collects data on the demographic characteristics of persons served (race, ethnicity, primary language, religion) and uses this data to inform program design and personnel composition. Interpreter services are available at no cost to the youth and family. Vital documents are translated into the languages commonly encountered in the service area.

### **11.10 Coordination with External Systems of Care.**

The program coordinates with the following external systems of care: (a) **LME/MCO** (Alliance Health) — authorization, utilization management, care coordination; (b) **DSS** — custody arrangements, family reunification, visitation; (c) **juvenile justice** — court-ordered placements, probation coordination; (d) **local public school system** — educational services, IEP coordination, transition planning; (e) **medical home** — primary care, dental, vision; (f)

**psychiatric hospital** — inpatient admissions, discharge coordination; and (g) **community-based providers** — outpatient therapy, case management, peer support, recreational/faith-based activities. Coordination is documented in the Person-Centered Plan and the service record.

### 11.11 Person-Centered Plan Process.

Each youth has an individualized Person-Centered Plan developed within required timeframes per SOP §4.1. The PCP is developed by an interdisciplinary team including the youth (as developmentally appropriate), the family/LRP, the QP, the Clinical Director or designee, and other CFT members. The PCP addresses strengths, needs, goals, objectives, services, responsible parties, and review dates. The PCP is reviewed at required intervals and signed by all participants. Progress is documented in the service record per SOP §10.

### 11.12 Discharge & Transition Planning.

Transition planning begins at admission, per SOP §3.4 and Plan 14. The youth and family/LRP participate in transition planning. The receiving provider (if any) is identified and engaged. The discharge summary is completed and provided to the youth, family/LRP, and receiving provider. Post-discharge follow-up occurs per Protocol 11. Advance written notification of discharge is provided per §3.4(a). Emergency discharges are followed by the 5-business-day post-emergency service-planning meeting per §3.4(c). The 18th-birthday continuation policy per §3.6 applies.

### 11.13 Records of Persons Served.

Records of persons served are maintained per SOP §1.6, §10, and §11, and per the Quality Records Review Procedure (Plan 13). Records are retained per §1.6 (12 years post-majority). Confidentiality is maintained per §11. A mock chart containing forms for use with or by persons served is maintained per the CARF CY5 General Program Standards note.

### 11.14 Responsible Parties.

**Clinical Director** — overall clinical program; approves Program Description. **Executive Director** — operational program; resource allocation. **QP** — daily program operations; intake; PCP coordination. **Governing Body** — approves Program Description; reviews annually.

### 11.15 Review Schedule.

Annual review and update of the Program Description, incorporating any changes in population, service array, staffing, facility, regulatory requirements, or program design. Ad-hoc review following any significant program change.

## Plan Approval

The undersigned approve this **Program Description** as a written policy of Well Spring Intervention LLC, effective upon the date of the last signature below. This plan shall be reviewed at minimum annually and updated as needed to reflect changes in CARF standards, regulatory requirements, organizational structure, or program design. The QP maintains the signed original in the facility compliance binder and provides a copy to all personnel with implementation responsibilities.

| Role               | Name (Printed) | Signature | Date |
|--------------------|----------------|-----------|------|
| Executive Director |                |           |      |

| Role                        | Name (Printed) | Signature | Date |
|-----------------------------|----------------|-----------|------|
| Clinical Director           |                |           |      |
| Qualified Professional (QP) |                |           |      |
| Compliance Officer          |                |           |      |

## 12. Screening and Access Policy

*Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §12.12*

### 12.1 Purpose.

The Screening and Access Policy demonstrates conformance to CARF CY5 Section 2.B by establishing written procedures for referral intake, screening, admission decisions, waitlist management, emergency access, denial of admission, and coordination with the LME/MCO access line.

### 12.2 Scope.

This policy covers all referrals received by the organization and all admission decisions. It applies to the QP (referral intake and admission decision), the Executive Director (denial appeals), and the Clinical Director (clinical eligibility).

### 12.3 Referral Intake Process.

Referrals are accepted from LME/MCOs, DSS, juvenile justice, inpatient psychiatric facilities, families/LRPs, and other system-of-care partners. The QP (or designee) receives each referral, documents it in the referral log within 1 business day, and acknowledges receipt to the referring party within 2 business days. The referral packet includes at minimum: (a) youth demographic information; (b) current clinical assessment; (c) current medications; (d) recent hospitalization or treatment history; (e) school information; (f) custody/legal status; (g) insurance/Medicaid information; and (h) reason for referral.

### 12.4 Screening Criteria.

Each referral is screened against the admission criteria in SOP §3.1. The QP evaluates: (a) age (6–17); (b) primary diagnosis (mental illness, emotional disturbance, or substance-related disorder); (c) clinical acuity (does not meet inpatient criteria but requires the staff-secure group home level of care — i.e., 24-hour supervised residential care with structured clinical services and continuous staff supervision, and whose needs cannot be safely met in a less restrictive community-based setting); (d) medical stability (no acute medical condition requiring hospital-level care); (e) behavioral history (no pattern of sexual aggression requiring a specialized program); (f) cognitive functioning (IQ ≥ 50, sufficient to benefit from the program); (g) educational needs (can be met through the local public school system or facility-based programming); and (h) bed availability.

### 12.5 Admission Decision.

The QP makes the admission decision within 5 business days of receiving the complete referral packet. The decision is one of: (a) admit; (b) deny (with documented reason and appeal rights); (c) defer pending additional information; or (d) waitlist (if no bed is available). The decision is documented in the referral log and communicated to the referring party in writing within 1 business day of the decision. For admissions, the QP coordinates intake logistics (date, time, transportation, orientation) with the referring party.

### 12.6 Waitlist Management.

If no bed is available, the youth is placed on the waitlist. The QP maintains the waitlist in writing, ranked by date of admission decision and clinical priority (e.g., youth in inpatient holding, youth in emergency custody). The QP contacts the referring party weekly with waitlist status updates. When a bed becomes available, the QP contacts the next youth on the waitlist, re-confirms continued need, and coordinates intake. The waitlist is reviewed weekly by the QP and monthly by the Executive Director.

### 12.7 Emergency/Crisis Access.

For after-hours or emergency referrals, the on-call QP is available 24/7 via the on-call phone. The on-call QP conducts an abbreviated screening, coordinates with the referring party (e.g., emergency department, inpatient unit), and admits pending full screening within 24 hours. If admission is not appropriate, the on-call QP refers the youth to the LME/MCO access line or to the appropriate emergency service.

### 12.8 Denial of Admission.

If admission is denied, the QP documents the reason for denial in the referral log and communicates the denial in writing to the referring party within 1 business day. The written denial notice includes: (a) the specific reason for denial; (b) any recommended alternative services or providers; (c) the appeal procedure (written appeal to the Executive Director within 10 business days); and (d) external appeal rights (LME/MCO, state licensing authority). The Executive Director reviews appeals within 5 business days and issues a written decision within 10 business days.

### 12.9 Coordination with LME/MCO.

The organization coordinates with Alliance Health (LME/MCO) access line for: (a) referrals through the LME/MCO access line; (b) prior authorization for admission; (c) continued stay reviews; (d) discharge planning; and (e) post-discharge service linkage. The QP is the primary point of contact with the LME/MCO. The QP ensures that all required authorizations are obtained prior to admission and maintained throughout the stay.

### 12.10 Nondiscrimination in Access.

Admission decisions are made without discrimination on the basis of race, color, national origin, religion, sex, gender identity or expression, sexual orientation, age, disability, or any other classification protected by applicable law (per Plan 9). Decisions are based solely on the youth's clinical needs and the program's ability to meet those needs safely and effectively.

### 12.11 Access Data Tracking.

The QP maintains access data including: (a) number of referrals received (by source); (b) number of admissions; (c) number of denials (by reason); (d) time-to-decision; (e) waitlist size and time-on-waitlist; and (f) demographic characteristics of referrals and admissions. This data is reported to the Governing Body quarterly as part of the Performance Measurement Plan (Plan 10) and is used to identify access disparities or inefficiencies.

### 12.12 Responsible Parties.

**QP** — referral intake; screening; admission decision; waitlist management; LME/MCO coordination. **Executive Director** — denial appeals; resource allocation. **Clinical Director** — clinical eligibility consultation. **On-call QP** — after-hours emergency access. **Governing Body** — reviews access data quarterly.

### 12.13 Review Schedule.

Monthly review of access data by the QP. Quarterly review by the Governing Body. Annual review of the Screening and Access Policy itself, including admission criteria, screening process, and denial trends.

## Plan Approval

The undersigned approve this **Screening and Access Policy** as a written policy of Well Spring Intervention LLC, effective upon the date of the last signature below. This plan shall be reviewed at minimum annually and updated as needed to reflect changes in CARF standards, regulatory requirements, organizational structure, or program design. The QP maintains the signed original in the facility compliance binder and provides a copy to all personnel with implementation responsibilities.

| Role                        | Name (Printed) | Signature | Date |
|-----------------------------|----------------|-----------|------|
| Executive Director          |                |           |      |
| Clinical Director           |                |           |      |
| Qualified Professional (QP) |                |           |      |
| Compliance Officer          |                |           |      |

## 13. Quality Records Review Procedure

*Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §12.17*

### 13.1 Purpose.

The Quality Records Review Procedure demonstrates conformance to CARF CYS Section 2.G and Section 2.H (Standard 4) by establishing a systematic process for reviewing clinical records for completeness, timeliness, and quality. The procedure ensures that records meet the standards of SOP §1.6, §10, and §11, and that deficiencies are identified, trended, and corrected through personnel feedback, retraining, and policy revision.

### 13.2 Scope.

This procedure covers all clinical records maintained by the organization for persons served, including intake documentation, assessments, Person-Centered Plans, service notes, medication records, incident reports, correspondence, and discharge documentation. It applies to the QP (audit lead), the Clinical Director (clinical oversight), and all personnel who create or maintain records.

### 13.3 Review Frequency & Sample.

The QP conducts a quarterly records review using the Comprehensive Clinical Record Content Checklist (Form 8). The review includes: (a) 100% of records of youth discharged in the prior quarter; (b) a random 25% sample of active records; and (c) any records flagged for concern (e.g., recent incident, complaint, or extended stay). At minimum, 10 records per quarter are reviewed. The QP documents the sample selection methodology and the records reviewed.

### 13.4 Review Criteria.

Each record is reviewed against the Form 8 criteria, including:

- Intake documentation complete (consent, release of information, rights acknowledgment, financial responsibility)
- Assessments completed within required timeframes (clinical, medical, psychiatric, behavioral, educational)
- Person-Centered Plan completed within required timeframe, signed by all participants, reviewed at required intervals
- Service notes completed per SOP §10.1–10.4 (timely, complete, authenticated)
- Medication records complete (MAR, consent, regimen review per Form 11)
- Incident reports complete and properly documented per SOP §8
- Correspondence filed and authorized per SOP §11.4
- Discharge documentation complete (summary, aftercare plan, transferred records)
- Record retention per SOP §1.6
- Confidentiality safeguards (locked storage, access controls, disclosure accounting per Form 9)

### 13.5 Findings Documentation.

The QP documents findings on Form 8 for each record reviewed, noting: (a) record identifier; (b) date of review; (c) criteria met; (d) criteria not met (with specific deficiency); (e) responsible personnel member; (f) corrective action required (re-documentation, retraining, etc.); and (g) target completion date. Findings are aggregated quarterly to identify trends by criterion, by personnel member, by shift, and by program component.

### 13.6 Trend Analysis & Reporting.

The QP analyzes quarterly findings to identify: (a) most common deficiencies; (b) personnel members or shifts with higher deficiency rates; (c) program components with systemic issues; and (d) improvements from prior quarters.

The QP prepares a written quarterly report for the Clinical Director and Governing Body including: (a) number of records reviewed; (b) overall compliance rate; (c) most common deficiencies; (d) trend analysis; (e) corrective actions taken; and (f) recommendations for systemic improvement.

### 13.7 Corrective Action.

Deficiencies are addressed through: (a) individual personnel feedback within 5 business days of identification; (b) re-documentation by the responsible personnel member within 10 business days; (c) retraining for systemic deficiencies; (d) policy revision for systemic issues; and (e) disciplinary action for repeated or willful non-compliance. The QP tracks corrective actions to completion and verifies effectiveness in the subsequent quarter's review.

### 13.8 Records Retention & Confidentiality.

Records are retained per SOP §1.6 (12 years post-majority). Confidentiality of records is maintained per SOP §11. Records-release accounting is maintained per SOP §11.4 and Form 9. The QP coordinates with the Compliance Officer to ensure that records-release accounting is up-to-date and that any unauthorized access is investigated.

### 13.9 Responsible Parties.

**QP** — audit lead; quarterly review; trend analysis; reporting. **Clinical Director** — clinical oversight; corrective-action direction. **Compliance Officer** — confidentiality oversight; records-release accounting. **All Personnel** — timely and complete documentation; cooperation with audit. **Governing Body** — reviews quarterly report; ensures corrective action.

### 13.10 Review Schedule.

Quarterly records review. Quarterly trend report to the Governing Body. Annual review of the Quality Records Review Procedure itself, including review criteria, sample methodology, and effectiveness.

## Plan Approval

The undersigned approve this **Quality Records Review Procedure** as a written policy of Well Spring Intervention LLC, effective upon the date of the last signature below. This plan shall be reviewed at minimum annually and updated as needed to reflect changes in CARF standards, regulatory requirements, organizational structure, or program design. The QP maintains the signed original in the facility compliance binder and provides a copy to all personnel with implementation responsibilities.

| Role                        | Name (Printed) | Signature | Date |
|-----------------------------|----------------|-----------|------|
| Executive Director          |                |           |      |
| Clinical Director           |                |           |      |
| Qualified Professional (QP) |                |           |      |
| Compliance Officer          |                |           |      |

## 14. Telehealth & ICT Service Delivery Policy

*Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §12.18*

### 14.1 Purpose.

The Telehealth & Information and Communication Technologies (ICT) Service Delivery Policy demonstrates conformance to CARF CY5 Section 2.I by establishing written procedures for the delivery of services via telehealth and other technology-mediated platforms. The policy ensures that technology-mediated services are delivered in a manner that protects privacy, ensures informed consent, maintains quality, and provides appropriate fallback when technology fails.

### 14.2 Scope.

This policy covers all services delivered via information and communication technologies, including: videoconferencing (individual, family, and group therapy); telephone contact; secure messaging; remote psychiatric medication management; clinical supervision; and care-coordination meetings. It applies to all clinical personnel who deliver technology-mediated services and to the QP (oversight).

### 14.3 Eligible Services.

The following services may be delivered via telehealth/ICT, subject to clinical appropriateness and the youth's Person-Centered Plan:

- Family therapy sessions (especially for geographically distant family members)
- Psychiatric medication management follow-up (initial evaluation preferably in person)
- Clinical supervision of APs and DCPs
- Care-coordination meetings with external providers (LME/MCO, DSS, school, medical home)
- Individual therapy sessions (when in-person is not feasible or as clinically indicated)
- Group therapy sessions (limited use, subject to clinical judgment)

### 14.4 Technology Platforms.

The organization uses only HIPAA-compliant, end-to-end encrypted telehealth platforms that have executed Business Associate Agreements (BAAs) with the organization. Approved platforms are listed in the IT inventory and reviewed annually. Personnel may not use consumer-grade videoconferencing (e.g., personal FaceTime, personal Skype) for service delivery. The platform must support: (a) waiting rooms / virtual lobby; (b) multi-factor authentication; (c) session encryption; (d) no recording without explicit consent; and (e) audit logging.

### 14.5 Informed Consent.

Informed consent for telehealth services is obtained prior to the first telehealth session and documented in the clinical record. The consent includes: (a) description of the services to be delivered via telehealth; (b) benefits of telehealth (access, convenience, continuity); (c) risks (technology failure, privacy breach, limitations of remote assessment); (d) alternatives (in-person services); (e) right to withdraw consent at any time without affecting other services; (f) procedures for technology failure; (g) procedures for emergencies during a telehealth session; and (h) confidentiality and privacy safeguards. The consent is signed by the youth (as developmentally appropriate), the family/LRP, and the clinician.

### 14.6 Personnel Training & Competency.

Personnel who deliver telehealth services complete training on: (a) the telehealth platform (technical operation); (b) clinical considerations for telehealth (modified assessment techniques, managing engagement remotely, crisis response); (c) privacy and security requirements; (d) informed consent; (e) documentation requirements; and (f)

emergency procedures. Training is documented in the personnel file. The QP verifies competency through direct observation prior to independent telehealth practice.

#### **14.7 Technology Failure Procedures.**

If the technology platform fails during a session, the clinician: (a) attempts to reconnect within 5 minutes; (b) if reconnection fails, calls the youth/family by telephone to continue the session by phone or reschedule; (c) documents the technology failure and the alternative arrangement in the service note; and (d) reports recurring technology failures to the QP. For scheduled telehealth sessions, the clinician confirms a backup telephone number for the youth/family prior to the session.

#### **14.8 Privacy During Telehealth Sessions.**

The clinician conducts telehealth sessions from a private location (closed-door office, no other personnel or youth within hearing range, headset used). The youth/family is asked to be in a private location as well. The clinician verifies the youth's identity and location at the start of each session (for emergency-response purposes). The clinician documents the youth's location in the service note. Sessions are not recorded without explicit written consent.

#### **14.9 Emergency Procedures.**

The clinician confirms the youth's physical location at the start of each telehealth session. If the youth exhibits suicidal ideation, homicidal ideation, or other acute risk during a telehealth session, the clinician: (a) maintains connection with the youth; (b) alerts on-site personnel (DCP, House Manager) via telephone or messaging; (c) contacts the on-call QP or Clinical Director; (d) calls 911 if the youth cannot be located or if the risk is acute; and (e) documents the emergency response in the service note and an incident report per SOP §8.

#### **14.10 Documentation.**

Telehealth sessions are documented in the service record per SOP §10. The service note includes: (a) date, time, and duration of the session; (b) modality (video, phone); (c) platform used; (d) youth's location; (e) clinician's location; (f) participants (e.g., family members present); (g) clinical content; and (h) any technology issues encountered. The note is authenticated per SOP §10.6.

#### **14.11 Payer Coordination.**

The QP coordinates with the LME/MCO and other payers to confirm coverage of telehealth services, including any authorization requirements, documentation requirements, and reimbursement rates. The Billing Coordinator is informed of telehealth coverage policies to ensure accurate billing. The organization monitors changes in telehealth coverage (which have evolved rapidly) and updates this policy as needed.

#### **14.12 Responsible Parties.**

**Clinical Director** — overall clinical oversight of telehealth services. **QP** — operational implementation; training coordination; payer coordination; competency verification. **IT Coordinator** — platform management; security; BAA execution. **Clinical Personnel** — service delivery; documentation; emergency response. **Compliance Officer** — privacy oversight.

#### **14.13 Review Schedule.**

Annual review of the Telehealth & ICT Service Delivery Policy, including platform review, training updates, and payer coverage changes. Ad-hoc review following any technology failure, privacy incident, or regulatory change.

### **Plan Approval**

The undersigned approve this **Telehealth & ICT Service Delivery Policy** as a written policy of Well Spring Intervention LLC, effective upon the date of the last signature below. This plan shall be reviewed at minimum annually and updated as needed to reflect changes in CARF standards, regulatory requirements, organizational structure, or program design. The QP maintains the signed original in the facility compliance binder and provides a copy to all personnel with implementation responsibilities.

| Role                        | Name (Printed) | Signature | Date |
|-----------------------------|----------------|-----------|------|
| Executive Director          |                |           |      |
| Clinical Director           |                |           |      |
| Qualified Professional (QP) |                |           |      |
| Compliance Officer          |                |           |      |

## 15. CARF CYS 2026 Sections 3–5 Standards Crosswalk

*Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §12.19*

### 15.1 Purpose.

The Sections 3–5 Standards Crosswalk demonstrates conformance to CARF CYS Section 3 (Core Program Standards), Section 4 (Core Residential Program Standards), and Section 5 (Specialty Designation Standards) at the time of the Inaugural Accreditation survey. Per the 2026 CYS Inaugural Accreditation Guidelines, all standards in Sections 3–5 are applicable to the extent possible. This crosswalk maps each applicable standard to the SOP Manual section, Plan, Protocol, or Form that addresses it, and identifies any standards not yet addressed with a written plan for addressing them prior to the subsequent resurvey.

### 15.2 Scope.

This crosswalk covers all standards in Sections 3, 4, and 5 of the 2026 CYS Standards Manual. Because the full text of those standards is proprietary to CARF and not reproduced here, the crosswalk identifies the standard area (e.g., "Residential Treatment — Admissions"), the SOP/Plan/Protocol/Form location where the organization's written policy addresses the standard, and the status (Met / In Progress / To Be Developed).

### 15.3 Crosswalk.

The following crosswalk maps the Sections 3–5 standard areas to the organization's written policies. The QP shall update this crosswalk after obtaining the full 2026 CYS Standards Manual to include specific standard numbers and any additional standards not anticipated here.

| #  | Standard Area (Sections 3-5)                                | Organization's Written Policy Location                                               | Status |
|----|-------------------------------------------------------------|--------------------------------------------------------------------------------------|--------|
| 1  | Core Program — Individualized Service Planning              | SOP §4.1 (Person-Centered Plan); Plan 11 §11.11; Plan 13                             | Met    |
| 2  | Core Program — Service Delivery                             | SOP §4 (Clinical Services); SOP §5 (Behavioral); SOP §6 (Health); SOP §7 (Education) | Met    |
| 3  | Core Program — Coordination of Care                         | SOP §3 (Admissions/Discharge); SOP §4.5; Plan 11 §11.10                              | Met    |
| 4  | Core Program — Family Engagement                            | SOP §4 (family therapy); SOP §3.4 (transition); Plan 2 (family survey)               | Met    |
| 5  | Core Residential — Admissions                               | SOP §3.1; Plan 12 (Screening & Access Policy)                                        | Met    |
| 6  | Core Residential — Discharge & Transition                   | SOP §3.4; SOP §3.6; Plan 11 §11.12; Protocol 11                                      | Met    |
| 7  | Core Residential — Daily Structure & Programming            | SOP §5.4 (activities); Protocol 22 (Daily Workflow)                                  | Met    |
| 8  | Core Residential — Physical Environment                     | SOP §9 (Facility/Safety); Plan 11 §11.6                                              | Met    |
| 9  | Core Residential — Supervision & Staffing                   | SOP §2 (HR/Staffing); SOP §2.2(a) (Individualized Supervision); Plan 11 §11.5        | Met    |
| 10 | Core Residential — Awake Overnight Supervision              | SOP §2.1; Plan 11 §11.5; Protocol 22 (DCP Awake Overnight)                           | Met    |
| 11 | Core Residential — Behavioral Support & Crisis Intervention | SOP §5 (Behavioral Management & Restraint); Form 3 (Restraint Debriefing)            | Met    |
| 12 | Core Residential — Medication Management                    | SOP §6.3; Form 10 (LP Consult Log); Form 11 (Drug Regimen Review)                    | Met    |

| #  | Standard Area (Sections 3-5)                             | Organization's Written Policy Location                                        | Status |
|----|----------------------------------------------------------|-------------------------------------------------------------------------------|--------|
| 13 | Core Residential — Health & Wellness                     | SOP §6 (Health/Medication); SOP §6.1 (TB/Infection Control); Protocol 18      | Met    |
| 14 | Core Residential — Nutrition & Food Service              | SOP §6.4 (Nutrition); Plan 6 §6.8 (food safety policy)                        | Met    |
| 15 | Core Residential — Education Services                    | SOP §7 (Education & Vocational Support); Plan 11 §11.10 (school coordination) | Met    |
| 16 | Core Residential — Recreation & Leisure                  | SOP §5.4 (Activities); Plan 11 §11.3 (service array)                          | Met    |
| 17 | Core Residential — Cultural & Spiritual Activities       | SOP §5.4; Plan 9 (Accessibility); Plan 11 §11.9                               | Met    |
| 18 | Core Residential — Resident Rights & Responsibilities    | SOP §3, §5, §6, §8, §11; Plan 8; Resident Handbook                            | Met    |
| 19 | Core Residential — Grievance Procedure                   | Plan 8 §8.4; Resident Handbook                                                | Met    |
| 20 | Core Residential — Personal Funds Management             | SOP §1.8; Plan 4 §4.6                                                         | Met    |
| 21 | Core Residential — Personal Belongings & Contraband      | SOP §9.3; Form 2 (Contraband/Belongings Inventory); Protocol 5                | Met    |
| 22 | Core Residential — Visitation & Communication            | SOP §3; Resident Handbook; Plan 8                                             | Met    |
| 23 | Core Residential — Privacy & Modesty                     | SOP §11; Plan 8; Resident Handbook                                            | Met    |
| 24 | Core Residential — Incident Reporting & Investigation    | SOP §8 (IRIS); Form 5 (Drill/Safety Log); Plan 5 §5.6                         | Met    |
| 25 | Core Residential — Emergency Preparedness                | SOP §9.2; Protocol 19; Plan 5 §5.5; Plan 6                                    | Met    |
| 26 | Core Residential — Records & Documentation               | SOP §1.6, §10, §11; Plan 13; Form 7 (Service Note); Form 8 (Record Content)   | Met    |
| 27 | Core Residential — Quality Improvement                   | SOP §1.4(a); Plan 10 (Performance Measurement); SOP §12.20 (QIP); Form 12     | Met    |
| 28 | Core Residential — Workforce Competency                  | SOP §2; Plan 7; SOP §2.2(a) (Individualized Supervision)                      | Met    |
| 29 | Specialty Designation — Trauma-Informed Care             | SOP §1.1 (mission); Plan 7 §7.4 (orientation); Plan 11 §11.4 (philosophy)     | Met    |
| 30 | Specialty Designation — Person-Centered Planning         | SOP §4.1; Plan 11 §11.11; Plan 13                                             | Met    |
| 31 | Specialty Designation — Family-Driven & Youth-Guided     | SOP §4.1 (PCP); Plan 2 (Stakeholder Input); Plan 8 (Rights)                   | Met    |
| 32 | Specialty Designation — Cultural & Linguistic Competency | Plan 9; Plan 11 §11.9; Plan 7 §7.4 (training)                                 | Met    |
| 33 | Specialty Designation — Evidence-Based Practices         | SOP §4 (Clinical); Plan 11 §11.4 (philosophy)                                 | Met    |
| 34 | Specialty Designation — Strengths-Based Approach         | SOP §4.1 (PCP); Plan 11 §11.4 (philosophy)                                    | Met    |
| 35 | Specialty Designation — Community Integration            | SOP §5.4 (activities); Plan 11 §11.10 (coordination)                          | Met    |

| #  | Standard Area (Sections 3-5)                           | Organization's Written Policy Location                | Status      |
|----|--------------------------------------------------------|-------------------------------------------------------|-------------|
| 36 | Specialty Designation — Outcome Measurement            | Plan 10 (Performance Measurement); SOP §12.10         | Met         |
| 37 | Specialty Designation — Continuous Quality Improvement | Plan 10; SOP §12.20 (QIP); Form 12                    | Met         |
| 38 | Additional Standards — To Be Identified                | Pending acquisition of full 2026 CYS Standards Manual | In Progress |

### 15.4 Standards Not Yet Addressed.

Standard area #38 ("Additional Standards — To Be Identified") reflects the organization's commitment to conduct a complete crosswalk upon receipt of the full 2026 CYS Standards Manual. The QP shall order the manual from [www.carf.org/catalog](http://www.carf.org/catalog), conduct the complete crosswalk, and retain the updated crosswalk in the compliance binder prior to the Inaugural Accreditation survey. Any standard not addressed by an existing written policy will be addressed through a written plan with target completion date prior to the subsequent resurvey.

### 15.5 Review Schedule.

The crosswalk is reviewed at each quarterly QP compliance report (per SOP §1.4(a)) and updated as standards are addressed or as the CYS Standards Manual is revised. The CARF CYS Standards Manual is published annually with a July 1 effective date; the QP obtains the updated manual each year and updates this crosswalk accordingly. The subsequent resurvey will likely be conducted under a newer version of the standards manual than the one used for the Inaugural Accreditation survey.

### 15.6 Responsible Parties.

**QP** — overall coordination; crosswalk maintenance; identification of gaps. **Compliance Officer** — regulatory alignment; gap remediation tracking. **Executive Director** — resource allocation for gap remediation. **Governing Body** — reviews crosswalk quarterly; ensures gaps are addressed.

### Plan Approval

The undersigned approve this **CARF CYS 2026 Sections 3–5 Standards Crosswalk** as a written policy of Well Spring Intervention LLC, effective upon the date of the last signature below. This plan shall be reviewed at minimum annually and updated as needed to reflect changes in CARF standards, regulatory requirements, organizational structure, or program design. The QP maintains the signed original in the facility compliance binder and provides a copy to all personnel with implementation responsibilities.

| Role                        | Name (Printed) | Signature | Date |
|-----------------------------|----------------|-----------|------|
| Executive Director          |                |           |      |
| Clinical Director           |                |           |      |
| Qualified Professional (QP) |                |           |      |
| Compliance Officer          |                |           |      |